

NIC CPD Micro-Credential: Palliative, End-of-Life & Bereavement Care for Professional Caregivers

Issuing organisation: National Institute of Caregivers Nigeria (NIC)

Credential type: Stand-alone Continuing Professional Development (CPD) Micro-Credential

Recommended duration: 5–6 CPD learning hours

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Scope statement: This is professional development for caregivers. It is not a medical palliative-care, nursing, hospice-clinician, prescribing, pain-management, prognosis, diagnosis or treatment qualification. It does not authorise a caregiver to diagnose, prescribe, alter medication, determine prognosis, make clinical end-of-life decisions, withdraw treatment or perform procedures outside scope. Clinical decisions must be made by appropriately authorised professionals through the applicable care-planning process.

1. Course title

NIC CPD Micro-Credential: Palliative, End-of-Life & Bereavement Care for Professional Caregivers

2. Course overview

This 5–6 hour CPD prepares professional caregivers to provide compassionate, dignified and person-centred support to people living with serious illness, advanced frailty, progressive conditions or approaching the end of life, and to support families before and after death.

Palliative care is an approach intended to improve quality of life and relieve suffering for people and families facing life-threatening illness; it includes physical, psychosocial and spiritual support and may be provided alongside disease-directed treatment rather than only in the final days ¹. End-of-life care overlaps with palliative care but refers more specifically to care during the final phase of life. Neither should be reduced to “doing tasks until death.”

The course focuses on comfort, dignity, presence, communication, symptom observation, basic non-invasive comfort measures within scope, emotional and family support, cultural

and spiritual sensitivity, advance-care awareness, professional boundaries, safeguarding, documentation, escalation and bereavement awareness.

When cure is no longer the goal, comfort, dignity and the person's wishes remain central to care.

Course at a glance

Element	Specification
Audience	Professional caregivers, healthcare assistants, home-care workers, residential-care staff, supervisors, community-care workers and other direct-care personnel
Duration	5–6 CPD learning hours
Structure	6 modules, 18 lessons, module assessments and a 35-question final assessment
Central capability	Compassionate, dignified, person-centred support within caregiver scope
Final pass mark	80% (28/35)
Scenario emphasis	28 of 35 final questions are scenario/application based
Practical limitation	Online completion does not prove clinical, medication, death-certification, counselling, hospice or emergency competence

3. Strategic purpose of the CPD

People approaching the end of life may experience pain or other symptoms, weakness, fatigue, reduced appetite, changing communication, fear, grief, dependence and loss of control. Families may experience anticipatory grief, guilt, anger, denial, conflict and exhaustion. Caregivers may be present at important moments and may notice changes before other team members do.

The purpose of this CPD is not to make caregivers clinicians. It is to help them ask **“What matters to this person?”** rather than only **“What tasks need to be completed?”** Learners will practise preserving dignity, providing appropriate comfort, communicating honestly, respecting choice, supporting families, recognising changes and escalating concerns.

The course complements NIC's elderly-care, dementia, nutrition, medication and psychosocial-support credentials. It does not repeat medication administration, nutrition therapy, dementia management or counselling. Instead, it brings these boundaries together in the context of serious illness, dying, death and bereavement.

4. Target audience

The target audience is professional caregivers, healthcare assistants, home-care workers, residential-care staff, care supervisors, community-care workers and other direct-care personnel who support people living with serious illness, advanced frailty, progressive conditions or approaching the end of life.

The course is not intended to qualify palliative-care physicians, hospice nurses, therapists, counsellors, grief specialists or clinicians. It does not replace local clinical training, emergency training, death-certification training or employer policy.

5. Recommended CPD hours

NIC should award **5–6 CPD learning hours**. A suggested allocation is 3.5–4 hours for lessons, 60–75 minutes for module assessments and 45–60 minutes for final assessment, reflection and review. Learning time should be based on meaningful engagement and assessment, not merely the browser being open.

6. Prerequisites

Learners should already understand basic caregiving, dignity, person-centred care, communication, observation, professional boundaries, confidentiality, safeguarding, documentation and reporting/escalation. Prior completion of a foundational caregiver programme, NIC elderly-care programme or equivalent direct-care experience is recommended.

7. Learning outcomes

By the end of the CPD, the learner should be able to:

1. Explain palliative care and distinguish it from end-of-life care at caregiver level.
2. Identify what matters to a person and preserve identity, dignity, privacy, values and choice.
3. Provide basic non-invasive comfort measures within the care plan and authorised role.
4. Observe and document possible changes in symptoms, breathing, responsiveness, mobility, appetite and communication.

5. Support food, fluids and mouth comfort without forcing intake or initiating clinical interventions.
6. Communicate sensitively with a person who is seriously ill or less responsive.
7. Respond appropriately to prognosis and treatment questions without giving clinical promises.
8. Recognise the purpose of advance-care planning without creating or interpreting legal directives.
9. Support family members while maintaining privacy, boundaries and role clarity.
10. Respect cultural and spiritual preferences while not permitting harmful practices.
11. Recognise safeguarding, coercion, exploitation and urgent deterioration concerns.
12. Provide bereavement-aware support without becoming a grief therapist.
13. Document objective observations and escalate clinical or safeguarding concerns.

8. Competency framework

Domain	Expected caregiver capability	Evidence
Palliative-care awareness	Understands comfort, quality of life and family support	Module 1
Person-centred care	Protects identity, dignity, privacy, preferences and relationships	Module 1
Comfort support	Provides authorised hygiene, mouth care, positioning, environment and presence	Module 2
Observation	Notices change without predicting death or diagnosing	Module 2
Communication	Listens, explains care, respects silence and refers clinical questions	Module 3
Autonomy	Respects choices and follows care planning when safety is affected	Module 3
Family support	Offers practical and emotional support without taking sides or giving prognosis	Module 4

Cultural/spiritual care	Asks, respects and escalates harmful practice	Module 4
Bereavement	Responds to grief with presence and appropriate referral	Module 5
Boundaries/safeguarding	Maintains role, privacy and protection from exploitation or abuse	Module 5
Documentation/escalation	Records facts and communicates urgent or meaningful concerns	Module 6

9. End-of-life response framework

The NIC COMFORT sequence

C — Clarify the person. Know identity, preferences, routines, relationships, values, faith, communication needs and the current care plan.

O — Observe dignity needs. Protect privacy, hygiene, clothing, positioning, environment, mouth comfort and familiar surroundings.

M — Make comfort safe. Provide non-invasive comfort measures within scope; never alter medication or perform unauthorised procedures.

F — Facilitate listening. Speak calmly, explain before care, allow silence and acknowledge emotion.

O — Observe changes. Notice symptom, breathing, responsiveness, mobility, appetite, fluid intake, distress and family concerns.

R — Relate with family. Listen, offer practical support, respect privacy and refer clinical questions.

T — Tell the team. Document and escalate changes, urgent symptoms, safeguarding concerns, family conflict or unavailable care-plan instructions.

Urgent override: If there is acute or life-threatening breathing difficulty, collapse, severe uncontrolled distress, suspected abuse, unsafe treatment or another emergency, **PROTECT SAFETY → ACTIVATE URGENT HELP → DOCUMENT → HAND OVER.** Do not wait for routine approval.

10. Complete module structure

Module	Title	Lessons	Assessment
1	Understanding Palliative and End-of-Life Care	1. What Palliative Care Means; 2. Person-Centred End-of-Life Care; 3. Dignity, Privacy and Comfort	6 questions
2	Observing Change and Providing Comfort	4. Observing Changes; 5. Food, Fluids and Mouth Comfort; 6. Pain and Symptom Awareness	6 questions
3	Communication, Choice and Advance-Care Awareness	7. Communicating with the Seriously Ill or Dying Person; 8. Difficult Conversations; 9. Autonomy and Advance-Care Awareness	6 questions
4	Family, Cultural and Spiritual Support	10. Supporting Families; 11. Family Conflict; 12. Cultural and Spiritual Care	7 questions
5	Death, Bereavement and Safeguarding	13. Death and Post-Death Care; 14. Bereavement Awareness; 15. Boundaries and Safeguarding	7 questions
6	Documentation, Escalation and Integrated Practice	16. Documentation and Handover; 17. Recognising Urgent Concern; 18. Integrated End-of-Life Judgement	7 questions

11. Detailed lesson outline

Lesson	Primary purpose
1	Understand palliative care and its relationship to end-of-life care
2	Preserve personhood, preferences and quality of life
3	Provide dignity and basic comfort within scope
4	Observe change without predicting death
5	Respond to appetite, fluids and mouth comfort appropriately
6	Recognise possible discomfort and report it
7	Communicate respectfully, including when responsiveness decreases
8	Respond to prognosis and difficult questions within role
9	Respect autonomy and recognise advance-care planning
10	Support families emotionally and practically
11	Remain neutral and safe during family disagreement
12	Respect cultural and spiritual preferences without imposing beliefs
13	Understand death and post-death boundaries
14	Provide bereavement-aware support
15	Maintain boundaries and recognise safeguarding concerns
16	Document facts and hand over clearly
17	Recognise urgent deterioration and escalate
18	Apply COMFORT in complex situations

12. Full lesson content

Module 1 — Understanding Palliative and End-of-Life Care

Lesson 1. What Palliative Care Means

Why this matters in professional caregiving

Palliative care helps people and families facing life-threatening illness by addressing quality of life and suffering. It may begin early and can be provided alongside treatments intended to slow or manage illness ¹. End-of-life care is a part of the broader palliative approach, usually focused on the period when a person is approaching death. They overlap but are not identical.

Learning objectives

The learner should be able to describe palliative care at caregiver level; distinguish it from end-of-life care; explain comfort and quality of life; and identify the caregiver's role.

Core instructional content

Palliative care is not “giving up,” and death is not a failure of caregiving. It may include symptom support, communication, psychosocial care, spiritual care, family support and coordination among professionals. A caregiver may help implement an authorised plan and provide presence, dignity and observation.

End-of-life care addresses the person's needs as death approaches. The person may have increased weakness, changing appetite, reduced communication or changing breathing, but individual experiences vary. Caregivers must not use a checklist to predict death or give families a prognosis.

The caregiver's role is to know the plan, preserve dignity, provide authorised comfort, observe, document and escalate. The caregiver does not diagnose, prescribe, determine prognosis, withdraw treatment or make clinical decisions.

Applying This in Practice

A person may receive palliative support at home, in a hospital, residential facility or community setting. In Nigeria, families may share care responsibilities across generations and locations. Respect family involvement while keeping the person's wishes and privacy central.

Case scenario

A relative says, “Palliative care means the doctors have abandoned him.”

Best response: Acknowledge the fear and explain that palliative care focuses on comfort, quality of life and support for the person and family; refer clinical questions to the appropriate professional.

Common mistakes / professional risks

Treating palliative care as hopelessness, giving prognosis, promising cure, confusing comfort with no care and speaking as if death is a caregiving failure.

Key practice points

1. Palliative care may begin before the final days.
2. It supports people and families.
3. End-of-life care overlaps but is narrower.
4. Comfort remains active care.
5. Caregivers observe and escalate; they do not make clinical decisions.

Lesson assessment

1. **MCQ:** Palliative care primarily aims to: A) abandon treatment; B) improve quality of life and relieve suffering; C) prescribe medication; D) certify death. **Answer: B. Objective: 1.**
2. **Scenario:** Relative says palliative care means giving up. **Answer:** Explain its comfort and quality-of-life purpose and refer clinical questions. **Objective: 1.**
3. **True/False:** Palliative care is limited to the last hours of life. **Answer: False. Objective: 1.**
4. **What should not happen?** Give a prognosis. **Answer:** Refer to authorised clinicians. **Objective: 1, 7.**
5. **Application:** Name three areas palliative care may address. **Answer:** Symptoms, psychosocial needs, spiritual needs, family support or quality of life. **Objective: 1.**
6. **Scope:** Can caregivers withdraw treatment? **Answer: No. Objective: 1.**

Lesson 2. Person-Centred End-of-Life Care

Why this matters in professional caregiving

Serious illness can make a person feel reduced to a diagnosis, bed, symptom or task. Person-centred end-of-life care protects identity, preferences, relationships, values, cultural identity and autonomy.

Learning objectives

The learner should be able to ask what matters; identify preferences; support meaningful routines; and avoid task-only care.

Core instructional content

Ask, “What matters to this person?” Consider preferred name, clothing, privacy, visitors, music, food, daily rhythm, personal belongings, language, faith, gender preferences and who the person wants involved. A person may still want ordinary conversation, humour, music, grooming, a favourite room or time with family.

Person-centred care does not mean every preference can be followed without consideration. The caregiver follows the plan and raises conflicts with the appropriate professional. Respecting choice is not abandoning safety.

Do not assume that a person who is less responsive has lost all dignity or should be discussed as if absent. Continue to introduce yourself, explain care and use respectful language.

Applying This in Practice

Use a “what matters” conversation early and update it as the person changes. In Nigerian settings, include family, faith and cultural preferences when the person wants them, while avoiding assumptions that all relatives or traditions are desired.

Case scenario

A client wants a particular radio programme and quiet time before visitors arrive, but staff prefer a fixed routine.

Best response: Explore whether the preference can be safely accommodated, preserve choice and raise practical barriers rather than dismissing it.

Common mistakes / professional risks

Task completion over personhood, infantilisation, public discussion, ignoring privacy, assuming family preference and imposing a routine.

Key practice points

1. Ask what matters.
2. Protect identity and relationships.
3. Offer choice where possible.
4. Explain care even when responsiveness decreases.

5. Follow plans and escalate conflicts.

Lesson assessment

1. **MCQ:** The person-centred question is: A) “What tasks are left?” ; B) “What matters to this person?” ; C) “What is easiest for staff?” ; D) “What does the family prefer?”
Answer: B. Objective: 2.
2. **Scenario:** Client prefers quiet before visitors. **Answer:** Explore and accommodate safely. **Objective: 2.**
3. **True/False:** Reduced responsiveness removes the need for respectful explanation.
Answer: False. Objective: 2, 6.
4. **What should not happen?** Discuss the person’s private condition loudly at the bedside. **Answer:** Protect privacy and dignity. **Objective: 2.**
5. **Application:** Name four preference areas. **Answer:** Name, clothing, visitors, music, food, routines, privacy, language or faith. **Objective: 2.**
6. **Scenario:** Family preference conflicts with documented client preference. **Answer:** Follow the plan and seek appropriate guidance. **Objective: 2, 9.**

Lesson 3. Dignity, Privacy and Comfort

Why this matters in professional caregiving

Dignity is protected through ordinary details: hygiene, clothing, positioning, mouth care, skin care, clean surroundings, temperature, privacy and respectful communication. These are active comfort measures, not minor tasks.

Learning objectives

The learner should be able to provide non-invasive comfort within the plan; protect privacy; and recognise interventions outside role.

Core instructional content

Comfort may include personal hygiene, clean clothing, appropriate bedding, repositioning according to the care plan, reducing noise, suitable lighting, temperature adjustment, gentle communication, familiar music, quiet presence and family connection. Mouth care should be performed only where trained and authorised.

Avoid unnecessary exposure. Close doors or curtains, explain what you are doing and use respectful language. Check consent where possible. Never force a position, perform invasive procedures, alter medication or apply treatments outside instruction.

Applying This in Practice

In home care, maintain familiar surroundings while reducing hazards. In facilities, protect privacy despite shared rooms. In hospitals, coordinate with nurses and clinicians rather than independently moving lines, tubes or equipment.

Case scenario

A client is uncomfortable, has dry lips and asks for help, but the caregiver is unsure whether mouth care is authorised.

Best response: Check the care plan and training; provide only authorised care and ask the responsible professional for clarification.

Common mistakes / professional risks

Rushing hygiene, exposing the person, moving equipment, applying unapproved substances, forceful repositioning and ignoring a request for comfort.

Key practice points

1. Comfort is active caregiving.
2. Protect privacy and explain care.
3. Follow the plan and training.
4. Do not perform invasive procedures.
5. Report unresolved discomfort.

Lesson assessment

1. **MCQ:** A caregiver-level comfort measure may be: A) altering medication; B) authorised repositioning and clean bedding; C) diagnosing pain; D) inserting a device. **Answer: B. Objective: 3.**
2. **Scenario:** Client requests mouth care but plan is unclear. **Answer:** Check authorisation and seek guidance. **Objective: 3.**
3. **True/False:** Privacy remains important during intimate care. **Answer: True. Objective: 3.**
4. **What should not happen?** Move clinical equipment independently. **Answer:** Seek trained assistance. **Objective: 3.**
5. **Application:** Name four comfort measures. **Answer:** Hygiene, bedding, environment, authorised positioning, mouth care, music or presence. **Objective: 3.**

6. **Documentation:** Record unresolved discomfort and who was informed. **Answer:** Correct. **Objective: 4, 13.**

Module 2 — Observing Change and Providing Comfort

Lesson 4. Observing Changes

Why this matters in professional caregiving

Caregivers may notice increased sleep, reduced appetite or fluids, breathing changes, reduced responsiveness, mobility change, skin change, weakness or reduced communication. Such observations can help the care team respond, but they do not prove that death is imminent.

Learning objectives

The learner should be able to observe change from baseline; avoid prediction; document facts; and report appropriately.

Core instructional content

Compare current presentation with usual pattern. Record time, context, what changed, associated signs and action taken. Avoid writing “actively dying” or “will die today” unless an authorised professional has documented that assessment and the record requires it.

Changes may reflect progression, medication, infection, pain, dehydration, distress or another cause. Sudden or severe change requires prompt review. Individual experiences vary; no checklist guarantees that death is imminent.

Applying This in Practice

Use an End-of-Life Observation Record during handover. In home care, contact the designated supervisor or clinical service. In facilities, follow the escalation pathway and do not rely on the next routine visit if the change is urgent.

Case scenario

A client is sleeping much more and eating little. The family asks, “Does this mean they will die tonight?”

Best response: Explain that changes can have different causes and that you are not qualified to predict; document and contact the appropriate clinician.

Common mistakes / professional risks

Predicting death, alarming families, dismissing change, diagnosing infection, recording vague impressions and delaying urgent review.

Key practice points

1. Compare with baseline.
2. Individual experiences vary.
3. Observe and report; do not predict.
4. Sudden severe change needs prompt escalation.
5. Document facts and context.

Lesson assessment

1. **MCQ:** Increased sleep near end of life is: A) a guaranteed sign of imminent death; B) a possible change requiring context and reporting; C) irrelevant; D) proof of medication error. **Answer: B. Objective: 4.**
2. **Scenario:** Family asks whether death will happen tonight. **Answer:** Do not predict; refer to the clinician. **Objective: 7.**
3. **True/False:** One sign guarantees death is imminent. **Answer: False. Objective: 4.**
4. **What should not happen?** Tell the family a precise timeline. **Answer:** Prognosis is outside caregiver scope. **Objective: 7.**
5. **Application:** Name five changes to observe. **Answer:** Sleep, intake, breathing, responsiveness, mobility, skin, weakness or communication. **Objective: 4.**
6. **Documentation:** Record baseline, current change, time, context and escalation. **Answer:** Correct. **Objective: 4, 13.**

Lesson 5. Food, Fluids and Mouth Comfort

Why this matters in professional caregiving

Appetite and thirst may change as illness progresses. Forcing food or fluid can cause distress and may be unsafe. Comfort-focused support must follow the care plan and respect the person's wishes.

Learning objectives

The learner should be able to support appropriate food and fluids; avoid force; provide authorised mouth comfort; and escalate changes.

Core instructional content

Offer food or fluids only as authorised and when the person is alert and able to accept safely. Respect refusal. Mouth comfort may include authorised oral care, moistening the mouth or lip care according to training and plan. Do not independently start artificial nutrition or hydration, change fluid restrictions or alter textures.

Decisions about clinically assisted nutrition or hydration belong to authorised clinical professionals and care-planning processes. The caregiver observes intake, distress, swallowing concern and comfort; documents and reports.

Applying This in Practice

Families may equate feeding with love and ask the caregiver to insist. Explain that comfort and safety matter and refer decisions to the clinical team. Coordinate with the Nutrition, Hydration & Mealtime Support CPD, but recognise that end-of-life decisions require the current plan.

Case scenario

A family member urges the caregiver to force water because the client has stopped drinking.

Best response: Do not force fluids. Check the plan, provide authorised mouth comfort, observe and contact the appropriate professional.

Common mistakes / professional risks

Force-feeding, rapidly giving fluids, changing texture, offering artificial hydration advice, ignoring refusal and treating intake as a performance target.

Key practice points

1. Follow the plan.
2. Respect refusal.
3. Mouth comfort may remain important.
4. Do not initiate clinical nutrition/hydration.
5. Observe, document and report.

Lesson assessment

1. **MCQ:** At end of life, food and fluid support should be: A) forced; B) guided by the plan, safety and the person's wishes; C) prescribed by caregivers; D) stopped automatically.

Answer: B. Objective: 5.

2. **Scenario:** Family asks for forced water. **Answer:** Do not force; follow plan and seek guidance. **Objective: 5.**
3. **True/False:** Caregivers may independently start artificial hydration. **Answer: False.** **Objective: 5.**
4. **What should not happen?** Change a fluid restriction without authorisation. **Answer:** Clinical decisions require the care team. **Objective: 5.**
5. **Application:** Name three observations. **Answer:** Intake, refusal, distress, swallowing, mouth comfort or responsiveness. **Objective: 5.**
6. **Documentation:** Record offer, acceptance/refusal, comfort measure and escalation. **Answer:** Correct. **Objective: 5, 13.**

Lesson 6. Pain and Symptom Awareness

Why this matters in professional caregiving

A person may not always be able to describe discomfort. Caregivers can notice facial expression, restlessness, moaning, withdrawal, guarding, movement change, sleep change or behaviour change.

Learning objectives

The learner should be able to recognise possible discomfort; use approved tools only when trained; provide basic comfort within scope; and report without diagnosing or altering medication.

Core instructional content

Observe what is different from usual and when it occurs. Non-verbal signs may indicate pain, breathlessness, anxiety, constipation, urinary difficulty, fatigue or another problem. Do not decide the cause from appearance alone. Ask simple questions when the person can respond and use approved tools only according to employer training.

Provide authorised non-invasive comfort measures and report. Do not give extra medication, change a dose, crush medication, stop medication or recommend a treatment unless authorised.

Applying This in Practice

Document exact signs and responses. “Restless, grimaced during repositioning and settled after authorised repositioning” is more useful than “in pain.”

Case scenario

A client grimaces and guards their abdomen. A family member asks the caregiver to give an extra pain tablet.

Best response: Do not alter medication. Provide authorised comfort, document signs and contact the nurse/clinician through the correct route.

Common mistakes / professional risks

Diagnosing pain cause, giving unapproved medication, minimising non-verbal signs, using a tool without training and failing to report.

Key practice points

1. Notice changes in expression and behaviour.
2. Do not diagnose the cause.
3. Use approved tools only when trained.
4. Never alter medication independently.
5. Observe, document and report.

Lesson assessment

1. **MCQ:** A possible sign of discomfort is: A) grimacing or guarding; B) a diagnosis; C) proof of a specific cause; D) irrelevant behaviour. **Answer: A. Objective: 4.**
2. **Scenario:** Family asks for extra medication. **Answer:** Do not alter medication; escalate. **Objective: 3, 4.**
3. **True/False:** Non-verbal signs can be meaningful observations. **Answer: True. Objective: 4.**
4. **What should not happen?** Record “pain due to cancer” without assessment. **Answer:** Record observed facts and refer. **Objective: 4.**
5. **Application:** Name four possible observations. **Answer:** Grimacing, moaning, restlessness, guarding, withdrawal, sleep/movement/behaviour change. **Objective: 4.**
6. **Documentation:** Record sign, timing, support, response and person informed. **Answer:** Correct. **Objective: 4, 13.**

Module 3 — Communication, Choice and Advance-Care Awareness

Lesson 7. Communicating with the Seriously Ill or Dying Person

Why this matters in professional caregiving

Communication remains important even when a person appears less responsive. Speaking calmly, introducing yourself and explaining care preserves dignity and may reduce fear.

Learning objectives

The learner should be able to introduce and explain care; allow silence; avoid infantilisation; and communicate respectfully when responsiveness changes.

Core instructional content

Use the person's preferred name. Say what you are doing before touching or moving them. Speak in a calm, ordinary adult voice. Allow time for response and respect silence. Do not assume that reduced responsiveness means the person cannot hear or deserves less explanation.

Observe facial expression, movement and other responses. If family members are present, protect the person from arguments, distressing speculation or unnecessary exposure.

Applying This in Practice

Use familiar language and authorised interpreters where needed. Cultural and spiritual preferences may affect who speaks, prays or visits; ask and follow the plan.

Case scenario

A caregiver speaks about a less-responsive client as if they are not present.

Best response: Address the client respectfully, explain care and remind colleagues to protect personhood and privacy.

Common mistakes / professional risks

Infantilisation, talking over the person, rushing, surprise touch, speaking about the person as absent and assuming hearing is absent.

Key practice points

1. Introduce yourself.
2. Explain before care.
3. Speak calmly and allow time.
4. Respect silence.
5. Never reduce personhood.

Lesson assessment

1. **MCQ:** A less-responsive person should be: A) ignored; B) addressed respectfully and informed before care; C) discussed publicly; D) infantilised. **Answer: B. Objective: 6.**
2. **Scenario:** Client does not answer. **Answer:** Continue respectful explanation and observe. **Objective: 6.**
3. **True/False:** Reduced response proves the person cannot hear. **Answer: False. Objective: 6.**
4. **What should not happen?** Talk over the client as if absent. **Answer:** Protect dignity. **Objective: 6.**
5. **Application:** Name three communication behaviours. **Answer:** Introduce, explain, calm voice, silence, time and adult language. **Objective: 6.**
6. **Documentation:** Record meaningful response or change. **Answer:** Correct. **Objective: 4, 13.**

Lesson 8. Difficult Conversations

Why this matters in professional caregiving

Clients may ask, “Am I dying?” , “How long do I have?” , “Will I get better?” or “Why is this happening?” A caregiver can be compassionate without providing a prognosis or false reassurance.

Learning objectives

The learner should be able to listen, acknowledge fear, avoid promises and refer clinical questions appropriately.

Core instructional content

Start with the emotion: “I can hear that you are worried.” Ask what the person most wants to know or how you can support them. Be honest about role: “I do not want to give information I am not qualified to provide, but I can help you speak with the nurse or doctor.”

Do not say “You will definitely get better,” “You only have days left,” or “Do not ask questions.” Do not impose personal beliefs. If the person expresses hopelessness, unbearable distress or immediate safety concern, follow the safeguarding and urgent escalation pathway.

Applying This in Practice

Families may ask the caregiver to hide information or promise a treatment outcome. Remain neutral, follow the care plan and involve the appropriate professional.

Case scenario

A client asks, “How long do I have?”

Best response: Acknowledge the worry, explain that prognosis is a clinical question and offer to contact the nurse or doctor.

Common mistakes / professional risks

False reassurance, prognosis, personal opinions, secrecy, avoiding the question entirely and dismissing emotion.

Key practice points

1. Listen before redirecting.
2. Acknowledge emotion.
3. Do not give prognosis.
4. Do not make clinical promises.
5. Refer appropriately.

Lesson assessment

1. **MCQ:** The best response to “How long do I have?” is: A) give a guess; B) acknowledge and refer to the clinician; C) change topic; D) promise recovery. **Answer: B. Objective: 7.**
2. **Scenario:** Family asks caregiver to hide information. **Answer:** Follow authorised communication and privacy processes; do not make promises. **Objective: 7, 9.**
3. **True/False:** Compassion requires giving a prognosis. **Answer: False. Objective: 7.**
4. **What should not happen?** Say “Everything will be fine” without basis. **Answer:** Use honest reassurance and referral. **Objective: 7.**
5. **Application:** Give one safe response. **Answer:** “I hear that you are worried; I can help you speak with the appropriate professional.” **Objective: 7.**
6. **Scenario:** Client expresses unbearable distress. **Answer:** Listen, protect safety and escalate. **Objective: 7, 11.**

Lesson 9. Autonomy and Advance-Care Awareness

Why this matters in professional caregiving

People may document or discuss preferences about treatment, place of care, decision-makers, comfort, spiritual wishes and end-of-life care. Caregivers should respect documented preferences but must not create or interpret legal directives beyond competence.

Learning objectives

The learner should be able to support choice; recognise advance-care planning; follow documented plans; and distinguish autonomy from abandoning safety.

Core instructional content

Advance-care planning is a process of discussing and recording preferences for future care if a person cannot communicate. It may include preferred decision-makers, place of care, treatments, comfort and spiritual wishes. Legal requirements vary, so caregivers must follow local policy and authorised professional guidance.

Respect choice about visitors, privacy, routines, belongings, food and spiritual practice where safe. If the person refuses care, do not force; report the refusal and follow the plan. If the choice creates serious safety risk, involve the appropriate professional rather than abandoning the person or making a unilateral decision.

Applying This in Practice

Do not sign as a witness, interpret a directive, decide capacity or tell a family that a preference is legally binding unless authorised. Report questions to the responsible professional.

Case scenario

A family asks the caregiver to decide whether hospital treatment should be stopped.

Best response: Explain that this is a clinical and care-planning decision outside role; contact the responsible professional and continue authorised support.

Common mistakes / professional risks

Interpreting legal documents, imposing beliefs, deciding capacity, forcing care, stopping treatment or taking sides in family debate.

Key practice points

1. Respect documented preferences.
2. Do not create or interpret legal directives.
3. Choice and safety must be considered through the care plan.

4. Do not make treatment decisions.
5. Escalate uncertainty.

Lesson assessment

1. **MCQ:** Advance-care planning concerns: A) future preferences and decision-making; B) caregiver prescribing; C) death certification; D) diagnosis. **Answer: A. Objective: 8.**
2. **Scenario:** Family asks caregiver to stop treatment. **Answer:** Do not decide; refer to responsible professionals. **Objective: 9.**
3. **True/False:** A caregiver may interpret a legal directive independently. **Answer: False. Objective: 8.**
4. **What should not happen?** Force care after refusal. **Answer:** Respect, document and seek guidance. **Objective: 9.**
5. **Application:** Name four possible preferences. **Answer:** Place of care, treatment, decision-maker, comfort, spiritual wishes, visitors or privacy. **Objective: 8.**
6. **Scenario:** Preference creates serious safety risk. **Answer:** Protect safety and involve the appropriate professional. **Objective: 9.**

Module 4 — Family, Cultural and Spiritual Support

Lesson 10. Supporting Families

Why this matters in professional caregiving

Families may experience fear, anticipatory grief, guilt, anger, denial, exhaustion and uncertainty. A caregiver can provide respectful presence and practical help without giving prognosis or taking over the family's role.

Learning objectives

The learner should be able to listen to families, communicate observations, protect privacy, maintain boundaries and escalate concerns.

Core instructional content

Listen without judging. Offer practical information within role: where to sit, who has been contacted, what routine support is authorised and what the next handover is. Do not interpret symptoms, promise outcomes or disclose information without consent and policy.

Families may disagree, blame themselves or ask the caregiver to make a decision. Remain neutral and involve the appropriate clinician, supervisor, safeguarding lead or social-

support professional.

Applying This in Practice

Family structures may be multigenerational, diaspora-connected or faith-based. Ask who the person wants involved. Do not assume the loudest relative is the decision-maker.

Case scenario

A family member asks, “Is she dying tonight?”

Best response: Acknowledge fear, explain that you cannot predict, share only authorised observations and contact the responsible professional.

Common mistakes / professional risks

Giving prognosis, taking sides, sharing private information, making promises, criticising family and becoming emotionally over-involved.

Key practice points

1. Listen and acknowledge.
2. Provide practical support within role.
3. Protect privacy.
4. Do not give prognosis.
5. Escalate conflict and safety concerns.

Lesson assessment

1. **MCQ:** Family support includes: A) prognosis; B) listening, practical support and appropriate referral; C) taking sides; D) clinical decisions. **Answer: B. Objective: 9.**
2. **Scenario:** Family asks for a timeline. **Answer:** Do not predict; refer. **Objective: 7, 9.**
3. **True/False:** The loudest relative is automatically the decision-maker. **Answer: False. Objective: 9.**
4. **What should not happen?** Share confidential information with every visitor. **Answer:** Follow consent and privacy policy. **Objective: 9.**
5. **Application:** Name three family emotions. **Answer:** Fear, guilt, anger, denial, exhaustion or uncertainty. **Objective: 9.**
6. **Scenario:** Family requests a clinical decision. **Answer:** Refer to authorised professionals. **Objective: 7, 9.**

Lesson 11. Family Conflict

Why this matters in professional caregiving

Conflict may arise when one relative wants aggressive treatment and another wants comfort-focused care, when family members argue near the client or when someone asks the caregiver to hide information. The caregiver must remain neutral and protect the person.

Learning objectives

The learner should be able to avoid taking sides; protect the client from distress; follow the plan; and escalate conflict.

Core instructional content

Do not mediate clinical disagreement or decide which relative is correct. Reduce distress around the client, use calm language and ask family to involve the appropriate professional. Continue authorised care and document relevant incidents.

If there is coercion, intimidation, financial exploitation, threats or unsafe interference with care, follow safeguarding. A family relationship does not remove risk.

Applying This in Practice

In home care, the caregiver may be physically present in family disputes. Maintain a safe position, do not disclose private information and contact the supervisor when conflict affects care.

Case scenario

Two relatives argue about hospital transfer at the bedside and ask the caregiver to support their preferred decision.

Best response: Remain neutral, protect the client from distress, follow the plan and contact the responsible professional.

Common mistakes / professional risks

Taking sides, giving clinical advice, confronting family, disclosing private information and ignoring coercion.

Key practice points

1. Protect the person from conflict.

2. Remain neutral.
3. Follow the plan.
4. Refer clinical disagreement.
5. Escalate safeguarding concerns.

Lesson assessment

1. **Scenario:** Family argues over treatment. **Answer:** Remain neutral and involve the authorised team. **Objective: 9, 11.**
2. **MCQ:** The caregiver should: A) choose a side; B) protect dignity and escalate; C) decide treatment; D) hide information. **Answer: B. Objective: 9.**
3. **True/False:** Family conflict can create a safeguarding concern. **Answer: True. Objective: 11.**
4. **What should not happen?** Give one relative confidential information to win an argument. **Answer:** Protect privacy. **Objective: 9.**
5. **Application:** Name three actions. **Answer:** Reduce distress, remain neutral, follow plan, document and report. **Objective: 9–11.**
6. **Scenario:** Relative threatens staff. **Answer:** Protect safety and escalate. **Objective: 11.**
7. **Documentation:** Record objective conflict and effect on care. **Answer:** Correct. **Objective: 13.**

Lesson 12. Cultural and Spiritual Care

Why this matters in professional caregiving

Religious and cultural practices can provide meaning, comfort, community and identity. Preferences may include prayer, music, family presence, traditional practice, burial expectations, language and gender considerations. People within the same community do not all want the same care.

Learning objectives

The learner should be able to ask about preferences; respect safe practices; avoid imposing beliefs; and escalate harmful practices.

Core instructional content

Ask what matters to the person and who they want involved. Support prayer, music, spiritual contact, family presence or culturally familiar routines where safe and authorised. Do not ridicule beliefs, provide spiritual counselling outside role or impose personal beliefs.

A practice should not be permitted simply because it is described as cultural if it creates serious harm, coercion, abuse or unsafe interference with care. Discuss concerns with the supervisor or safeguarding lead.

Applying This in Practice

Nigeria is religiously and culturally diverse. Use the person's own preferences and an authorised interpreter where needed. Consider privacy and gender preferences without assuming that every person wants same-gender care or family presence.

Case scenario

A family wants a traditional substance applied to a wound, but it is not in the care plan.

Best response: Do not apply it. Respectfully explain that it must be checked for safety and contact the appropriate professional.

Common mistakes / professional risks

Ridicule, cultural stereotyping, imposing beliefs, allowing harmful practice, unauthorised spiritual counselling and ignoring consent.

Key practice points

1. Ask rather than assume.
2. Respect safe preferences.
3. Do not impose beliefs.
4. Harmful practice is not protected by the label "cultural."
5. Escalate uncertainty.

Lesson assessment

1. **MCQ:** Cultural care begins with: A) assumption; B) asking the person; C) imposing the caregiver's faith; D) excluding family. **Answer: B. Objective: 12.**
2. **Scenario:** Family requests an unapproved traditional wound substance. **Answer:** Do not apply; seek professional guidance. **Objective: 12.**
3. **True/False:** Every person in a culture wants identical end-of-life practices. **Answer: False. Objective: 12.**
4. **What should not happen?** Ridicule a spiritual belief. **Answer:** Respect dignity and choice. **Objective: 12.**

5. **Application:** Name four preference areas. **Answer:** Prayer, music, family, language, privacy, gender, burial or ritual. **Objective: 12.**
6. **Scenario:** Spiritual leader requests private access. **Answer:** Follow consent, privacy and safeguarding policy. **Objective: 12.**
7. **Documentation:** Record preference and any safety concern. **Answer:** Correct. **Objective: 12, 13.**

Module 5 — Death, Bereavement and Safeguarding

Lesson 13. Death and Post-Death Care

Why this matters in professional caregiving

Death requires dignity, clear boundaries and correct procedure. Caregivers may support the environment and family, but formal certification of death and clinical decisions belong to authorised professionals.

Learning objectives

The learner should be able to recognise the caregiver's post-death role; preserve dignity; follow local procedure; and avoid declaring death independently.

Core instructional content

If a person appears unresponsive or has no obvious signs of life, follow the setting's emergency procedure. Do not assume death or wait if emergency response is required. A caregiver must not certify death, remove medical devices, transfer the body, release belongings or make legal determinations unless specifically trained and authorised.

After authorised confirmation, post-death care may include privacy, respectful covering, preparation of the environment, contacting designated people, protecting belongings and supporting family according to policy. Cultural or religious practice must be respected where safe and authorised.

Applying This in Practice

In homes, contact the designated service and follow its procedure. In facilities or hospitals, notify the nurse/clinical lead. Do not announce death publicly or discuss cause of death.

Case scenario

A client is unresponsive and a family member asks the caregiver to confirm that the person has died.

Best response: Do not certify death. Follow emergency or clinical notification procedure and preserve dignity while awaiting authorised assessment.

Common mistakes / professional risks

Declaring death, moving the body, removing devices, disclosing cause, handling belongings informally and ignoring emergency procedure.

Key practice points

1. Follow emergency procedure first.
2. Do not certify death.
3. Preserve dignity and privacy.
4. Follow authorised post-death procedure.
5. Respect culture within safety and policy.

Lesson assessment

1. **MCQ:** A caregiver may independently: A) certify death; B) follow notification procedure and preserve dignity; C) determine cause; D) remove devices. **Answer: B. Objective: 13.**
2. **Scenario:** Client appears unresponsive. **Answer:** Follow emergency/clinical notification procedure; do not assume or certify death. **Objective: 13.**
3. **True/False:** Post-death procedures vary by setting and policy. **Answer: True. Objective: 13.**
4. **What should not happen?** Remove equipment without authorisation. **Answer:** Await trained direction. **Objective: 13.**
5. **Application:** Name three dignity actions after authorised confirmation. **Answer:** Privacy, respectful covering, protecting belongings, family support or clean environment. **Objective: 13.**
6. **Scenario:** Family asks cause of death. **Answer:** Refer to authorised professional; do not speculate. **Objective: 7, 13.**
7. **Documentation:** Record observations, notifications and procedure followed. **Answer:** Correct. **Objective: 13.**

Lesson 14. Bereavement Awareness

Why this matters in professional caregiving

Bereavement affects people differently. Family members may feel sadness, anger, guilt, numbness, relief, confusion or spiritual distress. Caregivers can offer presence without becoming grief counsellors.

Learning objectives

The learner should be able to acknowledge loss; respect grief and cultural practice; recognise severe or unsafe distress; and refer appropriately.

Core instructional content

There is no single timetable or correct expression of grief. Avoid clichés such as “be strong,” “move on” or “everything happens for a reason.” Acknowledge the loss: “I am sorry. This is very difficult.” Listen if the person wants to speak and respect silence if they do not.

Notice when distress is severe, prolonged, worsening, associated with inability to maintain safety, self-harm concern or serious functional deterioration. Follow safeguarding and referral pathways. Do not diagnose complicated grief or promise that grief will follow a fixed course.

Applying This in Practice

Respect mourning, prayer, visits, burial and community practices where safe and authorised. Ask what support the person wants rather than assuming.

Case scenario

A bereaved family member says, “I cannot go on,” and appears unsafe.

Best response: Take the statement seriously, stay with the person where safe, activate urgent support according to local procedure and document objectively. Do not promise secrecy.

Common mistakes / professional risks

Clichés, rushing grief, imposing faith, diagnosing, ignoring safety and becoming the family’s sole support.

Key practice points

1. Grief varies.
2. Presence is valuable.
3. Respect cultural and spiritual practice.

4. Notice severe or unsafe distress.
5. Refer and escalate; do not provide therapy.

Lesson assessment

1. **MCQ:** A supportive bereavement response is: A) “Move on” ; B) “I’ m sorry; I’ m here to listen” ; C) “Do not cry” ; D) ignore. **Answer: B. Objective: 12.**
2. **Scenario:** Bereaved person says they cannot go on and appears unsafe. **Answer:** Take seriously and activate urgent support. **Objective: 11–12.**
3. **True/False:** Grief follows one fixed timetable. **Answer: False. Objective: 12.**
4. **What should not happen?** Promise secrecy about immediate danger. **Answer:** Explain limits and seek help. **Objective: 11.**
5. **Application:** Name three supportive actions. **Answer:** Listen, acknowledge, maintain presence, respect practice or connect to support. **Objective: 12.**
6. **Scenario:** Family wants privacy. **Answer:** Respect it while following safety and policy. **Objective: 12.**
7. **Documentation:** Record concern, exact words, action and referral. **Answer:** Correct. **Objective: 12–13.**

Lesson 15. Boundaries and Safeguarding

Why this matters in professional caregiving

End-of-life relationships can become emotionally intense. Families may offer gifts, ask for private contact or expect the caregiver to handle money, decisions or rituals. Boundaries protect the person, family and caregiver.

Learning objectives

The learner should be able to maintain professional boundaries; recognise exploitation or coercion; protect confidentiality; and escalate safeguarding concerns.

Core instructional content

Do not borrow or lend money, accept significant gifts, make secret arrangements, enter romantic or sexual relationships, use personal social media inappropriately, make private promises or take over family decisions. Use a respectful boundary statement and seek supervision.

Safeguarding concerns may include coercion, abuse, neglect, financial exploitation, intimidation, unsafe interference with treatment or pressure to hide information. Listen,

record facts and follow the pathway. Do not investigate or confront an alleged perpetrator alone.

Applying This in Practice

A caregiver may feel responsible for a grieving family. Support through the service, not through personal availability. If a family member demands private information, follow consent and privacy policy.

Case scenario

A relative offers the caregiver money to report that the client is comfortable when the client is distressed.

Best response: Refuse, record the concern, report safeguarding and document the client's actual condition.

Common mistakes / professional risks

Secret payments, falsified records, dual relationships, accepting pressure, gossip, confronting family and failing to report.

Key practice points

1. Boundaries remain important at end of life.
2. Never falsify comfort or symptoms.
3. Protect privacy.
4. Do not investigate alone.
5. Escalate coercion and exploitation.

Lesson assessment

1. **MCQ:** A safeguarding concern may include: A) financial coercion; B) a preferred song; C) a quiet visit; D) ordinary grief. **Answer: A. Objective: 11.**
2. **Scenario:** Relative offers money to alter documentation. **Answer:** Refuse, document and report. **Objective: 11, 13.**
3. **True/False:** End-of-life circumstances remove professional boundaries. **Answer: False. Objective: 11.**
4. **What should not happen?** Confront an alleged abuser alone. **Answer:** Follow safeguarding procedure. **Objective: 11.**

5. **Application:** Name four boundary risks. **Answer:** Money, gifts, secret contact, private decisions, social media or romantic relationship. **Objective: 11.**
6. **Scenario:** Family asks caregiver to hide symptoms. **Answer:** Document accurately and escalate. **Objective: 13.**
7. **Documentation:** Record objective facts and person informed. **Answer:** Correct. **Objective: 11, 13.**

Module 6 — Documentation, Escalation and Integrated Practice

Lesson 16. Documentation and Handover

Why this matters in professional caregiving

Accurate records enable continuity and appropriate assessment. At end of life, small changes may be important, and vague notes can delay support.

Learning objectives

The learner should be able to record observations, comfort measures, communication, family concerns, escalation and follow-up objectively.

Core instructional content

Record date, time, setting, baseline, observed change, exact words where important, food/fluid or mouth-care support, symptoms or behaviour, comfort measure, response, person contacted, advice and follow-up. Avoid unsupported diagnoses, prognosis, cause-of-death claims and labels.

A handover should state what changed, what helped, what remains concerning, what the plan says, what should not be done and who is following up. Urgent verbal escalation must occur in addition to the record.

Applying This in Practice

Use the End-of-Life Observation Record and Escalation Guide. Protect confidentiality and share only with authorised members of the care team.

Case scenario

A note says, “Client is dying and family is difficult.”

Best response: Replace or supplement with objective observations, exact family concern, actions taken and notifications through the approved process.

Common mistakes / professional risks

Vague labels, delayed reporting, backdating, deleting entries, personal messaging and recording a prediction instead of facts.

Key practice points

1. Record facts and context.
2. Include response and outcome.
3. Handover what to do and avoid.
4. Escalate urgently outside the form.
5. Protect confidentiality.

Lesson assessment

1. **MCQ:** Objective documentation is: A) “family difficult” ; B) “client grimaced during repositioning and settled after authorised repositioning” ; C) “client will die today” ; D) “client has terminal pain.” **Answer: B. Objective: 13.**
2. **Scenario:** Breathing changes are noted. **Answer:** Document and escalate according to urgency. **Objective: 4, 11, 13.**
3. **True/False:** A form always replaces urgent verbal escalation. **Answer: False. Objective: 13.**
4. **What should not happen?** Record a prognosis as fact. **Answer:** Record observations and refer. **Objective: 4, 7.**
5. **Application:** Name six record elements. **Answer:** Time, baseline, change, words, support, response, person contacted, advice or follow-up. **Objective: 13.**
6. **Scenario:** Family concern is omitted. **Answer:** Add objective concern and notify the appropriate person. **Objective: 10, 13.**

Lesson 17. Recognising Urgent Concern

Why this matters in professional caregiving

Some situations require immediate action: acute or life-threatening breathing difficulty, collapse, uncontrolled distress, severe new confusion, suspected abuse, unsafe treatment, serious allergic reaction or inability to maintain safety. The caregiver must not delay while seeking routine approval.

Learning objectives

The learner should be able to identify urgent situations; protect safety; activate emergency or clinical support; and document afterward.

Core instructional content

Remain calm, stay with the person where safe, call the designated emergency or clinical support, follow the local procedure and provide only trained actions. Do not diagnose, improvise medication, restrain without authority or leave a person in danger.

In a suspected death, follow emergency or clinical notification procedure. In a safeguarding concern, protect the person and report. In severe symptom change, contact the responsible clinician promptly.

NIC must insert current emergency contacts and setting-specific escalation routes before publication. A single universal pathway should not be assumed.

Applying This in Practice

In home care, keep emergency and supervisor contacts accessible. In residential or hospital settings, know the call system and clinical escalation route. During power or communication outages, use the alternative pathway.

Case scenario

A client develops sudden severe breathing difficulty and the supervisor does not answer.

Best response: Activate the local emergency response immediately; do not wait for routine supervisor approval.

Common mistakes / professional risks

Waiting, diagnosing, giving unapproved medication, searching for instructions during the emergency, leaving the person alone and failing to document.

Key practice points

1. Protect safety first.
2. Activate urgent help.
3. Follow trained and local procedure.
4. Do not manage alone.
5. Document and hand over afterward.

Lesson assessment

1. **MCQ:** Immediate severe breathing difficulty requires: A) routine approval; B) urgent emergency response; C) observation until tomorrow; D) family debate. **Answer: B.**
Objective: 11.
2. **Scenario:** Supervisor does not answer. **Answer:** Use emergency pathway without waiting. **Objective: 11.**
3. **True/False:** Caregivers should improvise medication in an emergency. **Answer: False.**
Objective: 3, 11.
4. **What should not happen?** Leave a person in danger to find routine paperwork.
Answer: Activate help and stay where safe. **Objective: 11.**
5. **Application:** Name four urgent situations. **Answer:** Severe breathing difficulty, collapse, uncontrolled distress, severe confusion, suspected abuse or unsafe treatment.
Objective: 11.
6. **Scenario:** Suspected abuse is disclosed. **Answer:** Protect, listen, document and escalate safeguarding. **Objective: 11, 13.**
7. **Documentation:** Record time, signs, actions, contacts and outcome. **Answer:** Correct.
Objective: 13.

Lesson 18. Integrated End-of-Life Judgement

Why this matters in professional caregiving

End-of-life care combines comfort, communication, family emotion, autonomy, culture, safety, boundaries and documentation. A simple framework helps the caregiver prioritise the person rather than the task.

Learning objectives

The learner should be able to apply COMFORT; prioritise dignity and safety; support family; avoid clinical decisions; and escalate.

Core instructional content

Clarify who the person is and what matters. Observe dignity needs. Make comfort safe within the plan. Facilitate listening. Observe changes. Relate respectfully with family. Tell the team through accurate documentation and escalation.

When a concern appears, ask: What is happening? What can I safely do? What must I not do? Is it urgent? What does the plan say? Who must be informed? Is this within my role?

Applying This in Practice

Use COMFORT during every shift and at handover. The framework is appropriate in homes, hospitals, facilities and community services.

Case scenario

A client is less responsive, breathing has changed, family members are arguing about treatment, a relative asks the caregiver to force fluids and another asks the caregiver to sign a decision.

Best response: Clarify and protect dignity; provide only authorised comfort; do not force fluids or sign/decide treatment; remain calm with family; follow the emergency or clinical pathway for breathing change; document and escalate all relevant concerns.

Common mistakes / professional risks

Treating reduced responsiveness as proof of death, forcing fluids, choosing a family side, signing decisions, predicting death, altering medication and failing to escalate.

Key practice points

1. Personhood comes before task completion.
2. Comfort must be safe and authorised.
3. Family support is not clinical decision-making.
4. Cultural respect does not permit harm.
5. Document and escalate.

Lesson assessment

1. **Integrated scenario:** Breathing changes and family conflict occur together. **Answer:** Protect safety, escalate clinical concern, remain neutral and document. **Objective: 4, 10–13.**
2. **MCQ:** The first COMFORT step is: A) decide treatment; B) clarify the person and plan; C) predict death; D) force intake. **Answer: B. Objective: 13.**
3. **True/False:** Family pressure authorises the caregiver to make treatment decisions. **Answer: False. Objective: 9.**
4. **What should not happen?** Force fluids or alter medication. **Answer:** Follow the plan and escalate. **Objective: 3, 5.**
5. **Application:** List COMFORT steps. **Answer:** Clarify, Observe dignity, Make comfort safe, Facilitate listening, Observe changes, Relate with family, Tell the team. **Objective: 13.**

6. **Scenario:** Client appears dead. **Answer:** Follow emergency/clinical notification procedure; do not certify. **Objective: 13.**
7. **Documentation:** Record observations, actions, family concerns, contacts and follow-up. **Answer:** Correct. **Objective: 13.**

13. Lesson assessments

Each lesson contains six or seven assessment items. NIC should provide immediate feedback explaining the professional reasoning. Items may be randomised while preserving the competency and safety standard.

14. Module assessments

Module 1 assessment

1. **Scenario:** Relative believes palliative care means abandonment. **Answer:** Explain comfort and quality-of-life purpose; refer clinical questions. **LO: 1.**
2. **MCQ:** Palliative care may occur alongside treatment. **Answer:** Correct. **LO: 1.**
3. **Scenario:** Client preference conflicts with staff routine. **Answer:** Explore safe accommodation. **LO: 2.**
4. **What should not happen?** Discuss the person publicly. **LO: 2–3.**
5. **True/False:** Reduced responsiveness removes dignity. **Answer:** False. **LO: 2.**
6. **Application:** Name authorised comfort measures. **Answer:** Hygiene, environment, bedding, positioning, mouth care or presence. **LO: 3.**

Module 2 assessment

1. **Scenario:** Increased sleep and reduced intake. **Answer:** Observe, document and report without predicting. **LO: 4.**
2. **MCQ:** No single sign guarantees imminent death. **Answer:** Correct. **LO: 4.**
3. **Scenario:** Family requests forced water. **Answer:** Do not force; follow plan and seek guidance. **LO: 5.**
4. **What should not happen?** Alter medication for grimacing. **LO: 6.**
5. **True/False:** Mouth comfort may remain important. **Answer:** True. **LO: 5.**
6. **Application:** Record exact observable signs. **Answer:** Correct. **LO: 4, 6.**

Module 3 assessment

1. **Scenario:** Less-responsive client. **Answer:** Continue respectful explanation. **LO:** 6.
2. **MCQ:** Prognosis questions should be referred to authorised clinicians. **Answer:** Correct. **LO:** 7.
3. **Scenario:** Family asks caregiver to stop treatment. **Answer:** Do not decide; refer. **LO:** 8–9.
4. **What should not happen?** Make a clinical promise. **LO:** 7.
5. **True/False:** Choice must be considered with safety and the care plan. **Answer:** True. **LO:** 9.
6. **Application:** Name advance-care preferences. **Answer:** Treatment, place, decision-maker, comfort, spiritual wishes or visitors. **LO:** 8.

Module 4 assessment

1. **Scenario:** Family asks for prognosis. **Answer:** Acknowledge and refer. **LO:** 9.
2. **MCQ:** Caregivers should remain neutral in family clinical conflict. **Answer:** Correct. **LO:** 10.
3. **Scenario:** Family requests unapproved traditional wound substance. **Answer:** Do not apply; seek guidance. **LO:** 12.
4. **What should not happen?** Ridicule a belief. **LO:** 12.
5. **True/False:** A family relationship removes safeguarding risk. **Answer:** False. **LO:** 10–12.
6. **Application:** Name three family supports. **Answer:** Listening, practical help, authorised communication, privacy and referral. **LO:** 9.
7. **Documentation:** Record conflict and effect on care objectively. **Answer:** Correct. **LO:** 13.

Module 5 assessment

1. **Scenario:** Client appears unresponsive. **Answer:** Follow emergency/clinical notification; do not certify. **LO:** 13.
2. **MCQ:** Grief varies between people. **Answer:** Correct. **LO:** 12.
3. **Scenario:** Bereaved person appears unsafe. **Answer:** Activate urgent support. **LO:** 11–12.
4. **What should not happen?** Accept money to alter records. **LO:** 11, 13.
5. **True/False:** End-of-life care removes boundaries. **Answer:** False. **LO:** 11.
6. **Application:** Name three post-death dignity actions after authorised confirmation. **Answer:** Privacy, respectful covering, belongings, environment or family support. **LO:** 13.

7. **Documentation:** Record exact words and escalation. **Answer:** Correct. **LO:** 12–13.

Module 6 assessment

1. **Scenario:** Breathing changes. **Answer:** Document and escalate according to urgency. **LO:** 4, 11, 13.
2. **MCQ:** Urgent help should not wait for routine approval. **Answer:** Correct. **LO:** 11.
3. **Scenario:** Family demands the caregiver decide treatment. **Answer:** Remain neutral and refer. **LO:** 7–9.
4. **What should not happen?** Record a prediction as fact. **LO:** 4, 13.
5. **True/False:** A form replaces urgent escalation. **Answer:** False. **LO:** 13.
6. **Application:** Apply COMFORT. **Answer:** Clarify, Observe dignity, Make comfort safe, Facilitate listening, Observe changes, Relate with family, Tell team. **LO:** 13.
7. **Integrated scenario:** Less responsiveness, family conflict and forced-fluid request. **Answer:** Protect dignity, do not force, remain neutral, escalate breathing/clinical concern and document. **LO:** 3–13.

15. Final assessment blueprint

The final assessment contains **35 questions**. The recommended pass mark is **80% (28/35)**. **28 questions are scenario/application based**, exceeding the required minimum of 23.

Content area	Questions	Number	Scenario/application
Palliative and end-of-life foundations	1–5	5	3
Person-centred care, comfort and observation	6–10	5	4
Communication, autonomy and advance-care awareness	11–15	5	4
Family, cultural and spiritual support	16–20	5	4
Death, bereavement and safeguarding	21–25	5	4

Documentation and urgent escalation	26–30	5	5
Integrated judgement	31–35	5	4
Total		35	28

16. Final assessment questions

1. **MCQ:** Palliative care aims to: A) abandon care; B) improve quality of life and relieve suffering; C) certify death; D) prescribe for all clients. **Answer: B. LO: 1.**
2. **Scenario:** Relative says palliative care means giving up. **Answer:** Explain comfort and support purpose and refer clinical questions. **LO: 1.**
3. **True/False:** Palliative care is limited to the last hours of life. **Answer: False. LO: 1.**
4. **Scenario:** Family asks caregiver for a precise death timeline. **Answer:** Do not predict; refer to the clinician. **LO: 1, 7.**
5. **What should not happen?** Treat death as a failure of caregiving. **Answer:** Comfort and dignity remain active care goals. **LO: 1–2.**
6. **Scenario:** Client has a preferred routine that staff find inconvenient. **Answer:** Explore safe accommodation and preserve choice. **LO: 2.**
7. **MCQ:** Dignity includes: A) privacy, identity and respectful communication; B) task speed only; C) public discussion; D) family control only. **Answer: A. LO: 2–3.**
8. **Scenario:** Client requests comfort but the intervention is outside the plan. **Answer:** Check authorisation and seek guidance. **LO: 3.**
9. **What should not happen?** Perform an invasive procedure without training. **Answer:** Stay within scope. **LO: 3.**
10. **Scenario:** Increased sleep and reduced appetite appear. **Answer:** Observe, document and report without predicting death. **LO: 4.**
11. **Scenario:** Client is less responsive. **Answer:** Continue respectful explanation and adult communication. **LO: 6.**
12. **MCQ:** When asked “How long do I have?” , the caregiver should: A) guess; B) acknowledge and refer; C) promise recovery; D) avoid all response. **Answer: B. LO: 7.**
13. **Scenario:** Family asks caregiver to hide information. **Answer:** Follow consent, privacy and authorised communication process. **LO: 7, 9.**

14. **True/False:** A caregiver may interpret an advance directive independently. **Answer:** False. **LO:** 8.
15. **Scenario:** Client refuses care. **Answer:** Respect, document, follow the plan and seek guidance if safety is affected. **LO:** 9.
16. **Scenario:** Family asks if the client will die tonight. **Answer:** Acknowledge fear and refer; do not predict. **LO:** 7, 10.
17. **MCQ:** Family support includes: A) prognosis; B) listening and practical support within role; C) taking sides; D) treatment decisions. **Answer:** B. **LO:** 10.
18. **Scenario:** Family argues at the bedside. **Answer:** Protect client from distress, remain neutral and involve the appropriate team. **LO:** 10.
19. **Scenario:** Family requests an unapproved traditional substance on a wound. **Answer:** Do not apply; seek guidance. **LO:** 12.
20. **What should not happen?** Ridicule a spiritual belief. **Answer:** Respect the person and their choices. **LO:** 12.
21. **Scenario:** Client appears unresponsive. **Answer:** Follow emergency/clinical notification procedure; do not certify death. **LO:** 13.
22. **MCQ:** Bereavement: A) has one fixed timetable; B) varies between people; C) should be rushed; D) is always a disorder. **Answer:** B. **LO:** 12.
23. **Scenario:** Bereaved person says, "I cannot go on," and appears unsafe. **Answer:** Take seriously and activate urgent support. **LO:** 11–12.
24. **Scenario:** Relative offers money to falsify comfort documentation. **Answer:** Refuse, document and report safeguarding. **LO:** 11, 13.
25. **What should not happen?** Promise secrecy about serious risk. **Answer:** Explain limits and escalate. **LO:** 11–12.
26. **Scenario:** Client develops severe breathing difficulty and supervisor does not answer. **Answer:** Activate urgent emergency response immediately. **LO:** 11.
27. **MCQ:** A useful record contains: A) prognosis; B) observations, action, response and notifications; C) gossip; D) diagnosis by caregiver. **Answer:** B. **LO:** 13.
28. **Scenario:** Family asks caregiver to alter medication. **Answer:** Do not alter; contact authorised professional. **LO:** 3, 11.
29. **True/False:** A form always replaces urgent verbal escalation. **Answer:** False. **LO:** 13.
30. **What should not happen?** Leave someone in immediate danger while seeking routine approval. **Answer:** Activate urgent help and stay where safe. **LO:** 11.

31. **Integrated scenario:** Less responsiveness, changed breathing, family conflict and forced-fluid request. **Answer:** Protect dignity, follow urgent pathway, do not force fluids or decide treatment, remain neutral, document and hand over. **LO:** 3–13.
32. **Scenario:** Family wants the caregiver to sign a treatment decision. **Answer:** Decline and refer to authorised professionals. **LO:** 8–10.
33. **Scenario:** Person's cultural preference conflicts with safety. **Answer:** Respect the meaning, do not permit harm and seek guidance. **LO:** 9, 12.
34. **Scenario:** Caregiver feels unable to cope emotionally after death. **Answer:** Maintain professional conduct, seek supervision and use staff-support pathway. **LO:** 11–13.
35. **Integrated scenario:** A person approaching death is uncomfortable, family members disagree, and a caregiver is asked for prognosis. **Answer:** Clarify the person and plan; provide authorised comfort; listen and acknowledge; remain neutral; do not predict or make treatment decisions; escalate symptoms and conflict; document facts, contacts and follow-up. **LO:** 1–13.

17. Answer key and explanations

Q	Answer	Professional reasoning
1	B	Palliative care actively relieves suffering and supports quality of life.
2	Explain and refer	It is not abandonment and clinical questions require professionals.
3	False	Palliative care can begin earlier and overlap with treatment.
4	Do not predict	Prognosis is outside caregiver scope.
5	Do not frame as failure	Comfort and dignity remain active care.
6	Explore safe accommodation	Person-centred care considers routine and choice.
7	A	Dignity includes privacy and identity.
8	Check authorisation	Comfort measures must be safe and within plan.

9	Do not perform it	Invasive procedures require training and authority.
10	Observe and report	Changes do not guarantee imminent death.
11	Explain respectfully	Reduced response does not remove personhood.
12	B	Empathy and appropriate referral are safe.
13	Follow authorised process	Privacy and communication require consent and policy.
14	False	Legal interpretation is outside role.
15	Respect and escalate	Choice is balanced with safety through the care plan.
16	Acknowledge and refer	Do not predict.
17	B	Caregivers provide listening and practical support, not prognosis.
18	Neutrality and protection	Conflict should not burden the client.
19	Do not apply	Unapproved substances may harm.
20	Do not ridicule	Spiritual identity deserves respect.
21	Follow procedure	Caregivers do not certify death.
22	B	Bereavement varies.
23	Urgent support	Unsafe distress requires escalation.
24	Refuse and report	Payment and falsification are safeguarding and professional concerns.
25	Do not promise secrecy	Serious safety information must be escalated.
26	Activate emergency response	Immediate danger takes priority.

27	B	Objective records support continuity and assessment.
28	Do not alter	Medication decisions belong to authorised professionals.
29	False	Urgent verbal escalation remains necessary.
30	Do not delay	Protect safety and activate help.
31	Integrated response	Safety, dignity, scope, family and documentation all matter.
32	Decline and refer	Caregivers do not make treatment decisions.
33	Respect and protect	Culture does not justify harm.
34	Supervision and staff support	Boundaries and wellbeing protect care quality.
35	COMFORT sequence	The response balances personhood, comfort, communication, family and scope.

18. Recommended behavioural competency verification

Online completion demonstrates knowledge and scenario judgement. It does not prove clinical palliative-care competence, medication management, death certification, counselling, grief therapy, prognosis communication or emergency competence.

Verification method	What it tests	Evidence
Comfort-care simulation	Privacy, hygiene, positioning, environment and presence	Assessor checklist
Difficult-conversation role-play	Listening, empathy, scope and referral	Communication rubric
Family-conflict simulation	Neutrality, privacy and escalation	Scenario record
Cultural/spiritual scenario	Respect, consent and safety	Structured observation

Post-death procedure simulation	Notification, dignity and role boundaries	Supervisor sign-off
Documentation exercise	Objective observation, handover and escalation	Marked record
Supervisor observation	Real-world compassion and professionalism	Signed verification

Critical-fail behaviour should include force-feeding or forced fluids, medication alteration, prognosis, death certification, unauthorised procedures, falsifying records, disclosing private information, taking sides in treatment decisions, delaying emergency help or allowing harmful practice without escalation.

19. Completion requirements

NIC should issue the credential when the learner has completed all 18 lessons, attempted all six module assessments, achieved at least **80% (28/35)** on the final assessment and acknowledged the scope statement. NIC may permit up to three final-assessment attempts, with review after failure.

The online credential demonstrates knowledge-based and scenario-based judgement in palliative and end-of-life principles, dignity, comfort, observation, communication, autonomy, advance-care awareness, family support, cultural and spiritual sensitivity, bereavement, boundaries, safeguarding, documentation and escalation. It does not demonstrate medical palliative-care, nursing, hospice-clinician, prescribing, pain-management, prognosis, death-certification, counselling or grief-therapy competence.

NIC should retain learner identity, enrolment, completion date, curriculum version, module and final scores, attempts, scope acknowledgement, certificate ID and behavioural-verification records where applicable. Records must be handled securely.

20. Certificate wording

Certificate title

NIC CPD Micro-Credential: Palliative, End-of-Life & Bereavement Care for Professional Caregivers

Competency statement

This credential recognises successful completion of NIC continuing professional development in palliative, end-of-life and bereavement care. The learner demonstrated

knowledge-based and scenario-based understanding of compassionate person-centred support, dignity, comfort, symptom observation, sensitive communication, autonomy, advance-care awareness, family support, cultural and spiritual sensitivity, death and post-death boundaries, bereavement awareness, safeguarding, documentation and escalation within caregiver scope.

The certificate should state the 5–6 CPD learning hours, completion date, certificate identifier and curriculum version. It must also state:

This CPD micro-credential is not a palliative-care physician, hospice-nursing, therapy, counselling, grief-specialist, prescribing, pain-management, clinical or death-certification qualification. It does not authorise the holder to determine prognosis, alter medication, make clinical end-of-life decisions, certify death or perform procedures outside authorised scope.

21. Digital badge specification

Field	Recommended value
Badge name	NIC CPD Palliative, End-of-Life & Bereavement Care
Description	Recognises professional development in compassionate, dignified and person-centred support for serious illness, dying, death and bereavement within caregiver scope.
Competencies	Palliative awareness; dignity; comfort; observation; communication; autonomy; family support; culture/spirituality; bereavement; safeguarding; documentation; escalation
CPD hours	5–6
Level	Professional CPD; intermediate workplace application
Evidence	18 lessons, six module assessments, final assessment at 80%, scope acknowledgement and behavioural verification where required
Verification	NIC verification URL/QR, certificate ID, date and curriculum version
Limitation	Not a clinical, prescribing, hospice, counselling, grief-specialist or death-

	certification qualification
Review period	Recommend review every 24 months or sooner when guidance, law, policy or local procedures change

22. NIC portal course description

Palliative, End-of-Life & Bereavement Care for Professional Caregivers is a 5–6 hour NIC CPD micro-credential for caregivers supporting people with serious illness, advanced frailty, progressive conditions or approaching the end of life.

Learners practise preserving dignity, providing comfort within scope, observing changes, supporting food and mouth comfort, communicating sensitively, respecting choices, understanding advance-care planning, supporting families, responding to cultural and spiritual preferences, recognising safeguarding concerns, providing bereavement-aware support and escalating clinical or safety concerns.

The course uses the **COMFORT** sequence: Clarify the person, Observe dignity needs, Make comfort safe, Facilitate listening, Observe changes, Relate with family and Tell the team. It does not qualify learners to diagnose, prescribe, determine prognosis, certify death, provide therapy or make clinical end-of-life decisions.

23. Recommended downloadable job aids

A. End-of-Life Comfort Checklist

Check	Yes / No / N/A	Notes
Current care plan checked		
Person’ s preferences and “what matters” known		
Privacy protected		
Hygiene and clothing supported		
Positioning checked according to plan		
Mouth comfort provided where authorised		

Environment comfortable and familiar		
Communication calm and respectful		
Family connection supported where authorised		
Discomfort or change documented and escalated		

B. Family Communication Guide

Use: “I can hear that this is frightening.” “I do not want to give information I am not qualified to provide.” “I can help you speak with the nurse or doctor.” “I will share only what I am authorised to share.” Avoid prognosis, false reassurance, taking sides, criticism, secrecy promises and clinical decisions.

C. End-of-Life Observation Record

Recommended fields: date; time; baseline; responsiveness; breathing change; movement; appetite/fluid change; facial or behavioural signs of discomfort; communication; comfort measures; response; family concern; person informed; urgency; follow-up.

D. Bereavement Support Guide

Acknowledge loss, listen, allow silence, respect culture and faith, avoid clichés, maintain appropriate presence, notice unsafe distress, share referral information through the authorised pathway and seek supervision. Do not diagnose or provide grief therapy.

E. End-of-Life Escalation Guide

Immediate emergency: acute breathing difficulty, collapse, inability to maintain safety, severe uncontrolled distress, suspected serious abuse or another life-threatening change. Activate emergency response.

Prompt clinical review: new or worsening discomfort, breathing change, marked responsiveness change, repeated vomiting, inability to swallow safely, sudden confusion, family interference with care or unavailable plan.

Safeguarding escalation: coercion, threats, financial exploitation, abuse, neglect, unsafe treatment or pressure to falsify records.

NIC must insert current Nigerian emergency contacts and setting-specific routes before launch.

F. Dignity & Choice Checklist

Check preferred name, privacy, clothing, visitors, language, faith, music, routines, belongings, food, spiritual support, decision-maker information and documented preferences. Record conflicts and escalate rather than deciding independently.

24. References

[1] World Health Organization, Palliative care.

[2] National Institute for Health and Care Excellence, End of life care for adults: service delivery.

[3] National Institute for Health and Care Excellence, Care of dying adults in the last days of life.

[4] World Health Organization, Integrating palliative care and symptom relief into primary health care.

[5] World Health Organization, Planning and implementing palliative care services: a guide for programme managers.

[6] World Health Organization, Guidance on community mental health services: promoting person-centred and rights-based approaches.

[7] National Institute for Health and Care Excellence, Care of dying adults in the last days of life: information for the public.

[8] Marie Curie, Information and support about end-of-life care and bereavement.

[9] International Council of Nurses, professional guidance and ethical resources.

Source-use statement: The references provide international and evidence-informed guidance. They are not Nigerian law, NIC accreditation rules or a substitute for local clinical, emergency, safeguarding, cultural, religious, privacy or employer procedures. NIC should verify links and insert Nigeria-specific contacts and requirements before publication.

25. Quality assurance review

Quality question	Finding	Action before launch
CPD value	Develops comfort, communication, family support, observation and	Pilot with working caregivers and supervisors.

	escalation beyond basic caregiving.	
Non-duplication	Complements NIC elderly-care, dementia, nutrition, medication and psychosocial-support credentials.	Cross-link rather than repeat full clinical content.
Dignity	Personhood, privacy, choice, relationships and “what matters” recur throughout.	Include a dignity audit in content review.
Comfort	Provides practical non-invasive support within plan and scope.	Add local comfort-care checklist and training requirements.
Communication	Includes less-responsive people, difficult questions and family conversations.	Pilot role-play where possible.
Scope	Prohibits diagnosis, prescribing, prognosis, treatment decisions and death certification.	Require scope acknowledgement.
Family support	Addresses fear, grief, guilt, conflict, privacy and neutrality.	Add local referral and family-support contacts.
Cultural relevance	Respects Nigerian diversity, faith, language, family and traditional practices without stereotyping.	Conduct local educator and community review.
Safeguarding	Includes coercion, abuse, exploitation, unsafe treatment and falsification.	Map the employer safeguarding pathway.
Death	Clearly distinguishes observation from formal confirmation and certification.	Insert setting-specific post-death procedure.
Bereavement	Provides useful awareness without making caregivers grief therapists.	Add staff-support and referral information.
Assessment quality	28/35 final questions test judgement and professional behaviour.	Retain reasoning feedback.
Practical value	Includes COMFORT, checklists, guides and observation record.	Make job aids downloadable.

Professional credibility	Includes competency framework, assessments, verification, certificate limits and review cycle.	Obtain NIC approval and employer consultation.
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Final implementation checklist

The NIC portal should follow **COURSE → MODULES → LESSONS → MODULE ASSESSMENTS → FINAL ASSESSMENT → CERTIFICATE**. Every lesson should display objectives, core content, scenario, professional risks, key practice points and assessment.

Before launch, NIC should add current emergency contacts; supervisor and clinical-lead contacts; palliative or hospice referral pathways; safeguarding contacts; post-death procedures; complaints routes; privacy requirements; cultural/spiritual support options; and alternative routes for outages or unavailable services.

The intended professional standard is:

I understand how to support a person with dignity and compassion when they are seriously ill or approaching the end of life. I can provide appropriate comfort within my role, communicate sensitively, respect choices and culture, support families, recognise important changes, maintain professional boundaries, document appropriately and know when to escalate.